

Advanced Non-Small Cell Lung Cancer: Oncologist Treatment Decisions

QUESTIONNAIRE REQUIREMENT DOCUMENT (QRE)

Field	Detail
Client	Meridian Therapeutics
Job number	MT-2026-208
Version	v3.1 FINAL, 30 May 2026
Supersedes	v3.0 of 22 May 2026. Changes marked in the version history
Author	Dr A. Raghavan, Associate Director, Healthcare
Reviewed by	K. Lindqvist, Research Director; S. Park, Analytics
Method	Online, physician panel, double opt-in
Markets	United States, Germany, Japan
Languages	English (US), German, Japanese
Target LOI	35 minutes
Sample size	n = 300 completes, 120 US, 90 DE, 90 JP
Fielding window	15 June to 24 July 2026
Complexity	High. Nested loops, MaxDiff, rotated profiles, interlocking quotas, adverse event reporting

Version history

Version	Date	Change	Requested by
v1.0	28 Apr 2026	First draft circulated for client review	Internal
v2.0	09 May 2026	Added KRAS segment throughout. Extended therapy list from 24 to 40 items	Client
v2.1	14 May 2026	Japan market added. Translation notes appended	Client
v3.0	22 May 2026	MaxDiff replaced the ranking exercise at Section F	Analytics
v3.1	30 May 2026	Profile D added. Loop cap raised from 2 to 3 therapies. Q47 wording amended after pilot	Client

Notes for the programmer

1. This study is subject to pharmacovigilance requirements. Any verbatim mentioning an adverse event must be flagged for review within 24 hours. See Appendix E.
2. Disable the back button for the whole survey. A respondent who exits mid-survey resumes at the start of the section they were in, not mid-loop.
3. Quotas are interlocking on specialty by market. Cells are defined in the quota table and must be checked at S1, S6 and again at completion.
4. All randomisation must be seeded per respondent and the seed stored, so that any respondent's survey can be exactly reconstructed for audit.
5. Store the display order of every rotated list, grid row set, grid column set and profile as separate export variables.
6. Terminations are of two kinds. A hard terminate ends the session immediately. A soft terminate completes the demographic block first and is coded separately. See Appendix D.
7. Response code values are identical across all three markets and must not be renumbered. Where wording differs by market, the variant is given in the question. See Appendix F.
8. Screen resolution below 1024 pixels wide must show a warning before Section G, which is not usable on a phone.
9. The therapy list at Q17 is long. Present it in two columns with an alphabetical jump index. Do not paginate it across screens.
10. Every numeric grid that must total 100 shows a live running total and blocks continuation until the total is exactly 100.
11. Soft checks show once. If the respondent confirms the entry, accept it and set the corresponding SOFTCHECK flag.

12. Capture time on page for every question in Sections F and G separately from overall LOI.

Interlocking quotas

Market	Specialty	Minimum	Maximum	Behaviour when full
US	Medical oncologist	80	90	Hard terminate, code 30
US	Pulmonologist treating NSCLC	30	40	Hard terminate, code 30
Germany	Medical oncologist	60	70	Hard terminate, code 30
Germany	Pulmonologist treating NSCLC	20	30	Hard terminate, code 30
Japan	Medical oncologist	60	70	Hard terminate, code 30
Japan	Pulmonologist treating NSCLC	20	30	Hard terminate, code 30
All	High volume, 20 or more patients a month	150	No cap	Monitor only
All	Academic setting	90	150	Monitor only
All	Community setting	120	No cap	Monitor only

Section A. Screener

ASK ALL

S1. What is your primary medical specialty?

Single code. Do not randomise. Apply the specialty by market quota here.

Medical oncology.....	1	
Pulmonology.....	2	
Radiation oncology.....	3	HARD TERMINATE, CODE 10
Thoracic surgery.....	4	HARD TERMINATE, CODE 10
Internal medicine.....	5	HARD TERMINATE, CODE 10
Haematology only.....	6	HARD TERMINATE, CODE 10
Palliative medicine.....	7	HARD TERMINATE, CODE 10
Another specialty.....	8	HARD TERMINATE, CODE 10

ASK ALL

S2. Are you board certified, or the local equivalent, in the specialty you selected?

Single code. GERMANY: replace board certified with Facharzt. JAPAN: replace with specialist certification from the relevant board.

Yes.....	1	
No, currently in training.....	2	HARD TERMINATE, CODE 11
No.....	3	HARD TERMINATE, CODE 11

ASK ALL

S3. In which year did you complete your specialty training?

Numeric entry, four digits. Valid range 1975 to 2024. Derive YEARS_PRACTICE = 2026 minus S3.

IF YEARS_PRACTICE < 2, HARD TERMINATE, CODE 12.

IF YEARS_PRACTICE > 40, HARD TERMINATE, CODE 12.

ASK ALL

S4. In which type of setting do you spend the majority of your clinical time?

Single code. Apply the setting quota as monitor only.

Academic or university teaching hospital.....	1	
Comprehensive cancer centre.....	2	
Community hospital.....	3	
Community oncology practice or clinic.....	4	
Private practice, single specialty.....	5	
Private practice, multi specialty.....	6	
Other setting.....	7	HARD TERMINATE, CODE 17

ASK ALL

S5. Do you personally select and prescribe systemic therapy for patients with advanced NSCLC?

Single code.

Yes, I am the primary decision maker.....	1	
Yes, I share the decision within a tumour board.....	2	
I recommend but another physician prescribes.....	3	HARD TERMINATE, CODE 14
No, I refer these patients onward.....	4	HARD TERMINATE, CODE 14

ASK ALL

S6. Approximately how many patients with advanced or metastatic non-small cell lung cancer do you personally manage in a typical month?

Numeric entry. Valid range 0 to 200. Apply the volume quota here.

IF S6 < 5, HARD TERMINATE, CODE 13.

If S6 > 120, show a soft check: You have entered a figure above 120 patients a month, please confirm this is correct. Set SOFTCHECK_S6 if confirmed.

ASK IF S1 = 2, PULMONOLOGY

S7. Of the advanced NSCLC patients you see, for approximately what proportion do you personally initiate systemic therapy rather than referring to a medical oncologist?

Numeric, 0 to 100. This question exists to exclude pulmonologists who diagnose but do not treat.

IF S7 < 25, HARD TERMINATE, CODE 14.

ASK ALL

S8. Are you, or is a member of your immediate household, employed by any of the following?

Multi code. RANDOMISE codes 1 to 10. Anchor code 11 last, exclusive.

- | | | |
|--|----|--------------------------------|
| A pharmaceutical, biotechnology or medical device company..... | 1 | HARD TERMINATE, CODE 15 |
| A contract research organisation..... | 2 | HARD TERMINATE, CODE 15 |
| A market research agency or consultancy..... | 3 | HARD TERMINATE, CODE 15 |
| An advertising or medical communications agency..... | 4 | HARD TERMINATE, CODE 15 |
| A health technology assessment body..... | 5 | HARD TERMINATE, CODE 15 |
| A national or regional regulatory authority..... | 6 | HARD TERMINATE, CODE 15 |
| A public relations firm serving healthcare clients..... | 7 | HARD TERMINATE, CODE 15 |
| A specialty pharmacy or pharmacy benefit manager..... | 8 | HARD TERMINATE, CODE 15 |
| An insurance company or payer organisation..... | 9 | HARD TERMINATE, CODE 15 |
| A journalism or publishing organisation covering healthcare..... | 10 | HARD TERMINATE, CODE 15 |
| None of these..... | 11 | |

ASK ALL

S9. Have you taken part in any market research study about lung cancer treatment in the past three months?

Single code.

- | | | |
|-----------------------------------|---|--------------------------------|
| Yes, in the past month..... | 1 | HARD TERMINATE, CODE 18 |
| Yes, one to three months ago..... | 2 | |
| No..... | 3 | |

ASK ALL

S10. Please confirm that you agree to take part in this research and that your responses may be used in aggregate for research purposes.

Single code. Compulsory. GERMANY and JAPAN: use the approved local consent wording supplied separately, do not translate this text directly.

- | | | |
|---------------------|---|--------------------------------|
| I agree..... | 1 | |
| I do not agree..... | 2 | HARD TERMINATE, CODE 19 |

Section B. Practice profile

ASK ALL WHO QUALIFY

Q1. Thinking of all the advanced NSCLC patients you manage in a typical month, approximately what percentage falls into each of the following groups?

Numeric grid. Each row 0 to 100. The column must total exactly 100 before the respondent can continue. Show a running total. Do not randomise rows, this is a clinical sequence.

Row	Patient group	Variable
a	EGFR mutation positive, common mutations	Q1a
b	EGFR mutation positive, exon 20 insertion	Q1b
c	ALK rearrangement positive	Q1c
d	ROS1 rearrangement positive	Q1d
e	KRAS G12C mutation positive	Q1e
f	MET exon 14 skipping	Q1f
g	RET fusion positive	Q1g
h	Other actionable driver alteration	Q1h
i	PD-L1 50 percent or above, no driver alteration	Q1i
j	PD-L1 1 to 49 percent, no driver alteration	Q1j
k	PD-L1 below 1 percent, no driver alteration	Q1k
l	Biomarker status unknown or not tested	Q1l

ASK ALL

Q2. And approximately what percentage of these patients are you treating in each line of therapy in a typical month?

Numeric grid, rows are first line, second line, third line or later, maintenance. Must total exactly 100.

ASK ALL

Q3. In which lines of therapy do you personally make treatment decisions?

Multi code, minimum one selection.

First line.....1
Second line.....2
Third line or later.....3
Maintenance therapy.....4
Adjuvant or neoadjuvant.....5

ASK ALL

Q4. How many new advanced NSCLC patients do you start on systemic therapy in a typical month?

Numeric, 0 to 100. Soft check if the value exceeds 56.

ASK ALL

Q5. Does your institution hold a regular multidisciplinary tumour board for thoracic malignancies?

Single code.

Yes, weekly.....1
Yes, every two weeks.....2
Yes, monthly.....3
Yes, ad hoc only.....4
No.....5

ASK IF Q5 = 1 TO 4

Q6. For approximately what proportion of your advanced NSCLC patients is the treatment decision taken at the tumour board rather than by you alone?

Numeric, 0 to 100.

Section C. Diagnostic and biomarker testing

ASK ALL

Q7. Which of the following tests are routinely performed for your advanced NSCLC patients before first-line treatment is selected?

Multi code. RANDOMISE codes 1 to 15. Anchor code 16 last, exclusive.

Single-gene EGFR test.....	1
Single-gene ALK test.....	2
PD-L1 immunohistochemistry.....	3
Small targeted NGS panel, fewer than 50 genes.....	4
Large targeted NGS panel, 50 to 500 genes.....	5
Whole exome sequencing.....	6
Whole genome sequencing.....	7
RNA fusion panel.....	8
Liquid biopsy, circulating tumour DNA.....	9
Tumour mutational burden.....	10
Microsatellite instability.....	11
Fluorescence in situ hybridisation.....	12
Polymerase chain reaction hotspot panel.....	13
Immunohistochemistry panel beyond PD-L1.....	14
Methylation profiling.....	15
None of these are routine.....	16

ASK ALL

Q8. For approximately what proportion of your advanced NSCLC patients is comprehensive biomarker testing completed before first-line therapy begins?

Numeric, 0 to 100. Store as BIOMARKER_COMPLETE.

ASK IF Q8 < 80

Q9. What prevents comprehensive biomarker testing being completed before treatment in the remaining patients?

Multi code, maximum four. RANDOMISE codes 1 to 9. Anchor code 10 last.

Turnaround time is too slow to guide treatment.....	1
Insufficient tissue from the initial biopsy.....	2
Testing is not reimbursed for all patients.....	3
No local laboratory capability, samples must be sent out.....	4
Patient is too unwell to wait for results.....	5
Testing is not part of our institutional pathway.....	6
Cost to the patient.....	7
Uncertainty about which panel to order.....	8
Results arrive but are difficult to interpret.....	9
Something else.....	10

ASK ALL

Q10. What is the typical turnaround time, in calendar days, from biopsy to receiving comprehensive biomarker results?

Numeric, 1 to 90. Add a Don't know checkbox coded 98 which clears the numeric field.

ASK ALL

Q11. How often do you begin treatment before biomarker results are available?

Single code, 5-point frequency scale.

Always.....	1
Often.....	2
Sometimes.....	3
Rarely.....	4
Never.....	5

ASK IF Q11 = 1 TO 3

Q12. When you begin treatment before results are available, what do you typically start?

Open text, minimum 10 characters.

Section D. Therapy awareness and use

ASK ALL

Q13. Which of the following therapies are you aware of for the treatment of advanced NSCLC?

Multi code. RANDOMISE codes 1 to 40. Anchor code 41 last, exclusive. Present in two columns with an alphabetical jump index. Store the randomised order as ORDER_Q13 and reuse it at Q14, Q15 and Q16.

Pembrolizumab (Keytruda).....	1
Nivolumab (Opdivo).....	2
Atezolizumab (Tecentrig).....	3
Durvalumab (Imfinzi).....	4
Cemiplimab (Libtayo).....	5
Ipilimumab (Yervoy).....	6
Tremelimumab (Imjudo).....	7
Osimertinib (Tagrisso).....	8
Gefitinib (Iressa).....	9
Erlotinib (Tarceva).....	10
Afatinib (Gilotrif).....	11
Dacomitinib (Vizimpro).....	12
Amivantamab (Rybrevant).....	13
Mobocertinib (Exkivity).....	14
Lazertinib (Lazcluze).....	15
Alectinib (Alecensa).....	16
Brigatinib (Alunbrig).....	17
Lorlatinib (Lorbrena).....	18
Ceritinib (Zykadia).....	19
Crizotinib (Xalkori).....	20
Entrectinib (Rozlytrek).....	21
Repotrectinib (Augtyro).....	22
Selpercatinib (Retevmo).....	23
Pralsetinib (Gavreto).....	24
Capmatinib (Tabrecta).....	25
Tepotinib (Tepmetko).....	26
Sotorasib (Lumakras).....	27
Adagrasib (Krazati).....	28
Dabrafenib plus trametinib (Tafinlar plus Mekinist).....	29
Larotrectinib (Vitrakvi).....	30
Trastuzumab deruxtecan (Enhertu).....	31
Ramucirumab (Cyramza).....	32
Bevacizumab (Avastin).....	33
Pemetrexed (Alimta).....	34
Carboplatin.....	35
Cisplatin.....	36
Paclitaxel or nab-paclitaxel.....	37
Docetaxel.....	38
Gemcitabine.....	39
Meridian compound MT-4410.....	40
None of these.....	41

HARD TERMINATE, CODE 16

ASK IF AWARE OF AT LEAST ONE THERAPY AT Q13

Q14. And which of these have you personally prescribed in the last 12 months?

Multi code. PIPE IN only therapies selected at Q13, in ORDER_Q13. Add code 41, None of these, anchored and exclusive.

IF Q14 = 41 ONLY, SKIP TO SECTION F.

ASK IF AT LEAST ONE THERAPY PRESCRIBED AT Q14

Q15. Of the therapies you have prescribed, approximately what share of your advanced NSCLC patients currently receives each?

Numeric grid. Rows are the therapies selected at Q14, in ORDER_Q13. Each row 0 to 100. Total must not exceed 100 but need not equal it, since patients may receive none of these. Show a running total.

Derive TOP3 = the three therapies with the highest share at Q15, breaking ties by ORDER_Q13. If fewer than three have a share above zero, TOP3 contains only those above zero. Store as TOP3_1, TOP3_2, TOP3_3 and LOOPCOUNT.

ASK ALL WHO REACH HERE

Q16. For each of the following patient segments, which therapy would you most often select in the first line today?

Grid, one single-code dropdown per row. ROWS are only the segments where the respondent entered a value above zero at Q1. Dropdown contains the therapies selected at Q13 in ORDER_Q13, plus a No systemic therapy option coded 97 and a Not applicable option coded 98.

If more than eight rows qualify, split across two screens at the natural break between driver-positive and driver-negative segments.

Section E. Therapy loop

LOOP INSTRUCTION. The block Q17 to Q24 is asked once for each therapy in TOP3, in descending share order. The loop runs a minimum of one and a maximum of three times. Store the loop iteration number and the therapy code on every question in the block. Piped text [THERAPY] refers to the therapy for the current iteration.

A respondent must complete all iterations. Do not allow the loop to be exited early. Show progress as, for example, Therapy 2 of 3.

Rotation orders established inside the loop must be held constant across iterations so that ratings remain comparable.

ASK FOR EACH THERAPY IN TOP3

Q17. In which line or lines of therapy do you most often use [THERAPY]?

Multi code. Show only lines selected at Q3.

ASK FOR EACH THERAPY IN TOP3

Q18. For which patient segments do you most often select [THERAPY]?

Multi code. Show only the segments where the respondent entered a value above zero at Q1. If only one segment qualifies, auto-punch it and skip this question for that iteration.

ASK FOR EACH THERAPY IN TOP3

Q19. How would you rate [THERAPY] on each of the following?

Grid. ROTATE rows, hold the order constant across iterations. Scale 1 to 10 where 1 is Very poor and 10 is Excellent. Add a Cannot rate option coded 98, do not rotate it.

Row	Dimension
a	Efficacy in the line where you use it most
b	Durability of response
c	Overall tolerability
d	Manageability of adverse events when they occur
e	Ease of administration for the clinic
f	Convenience for the patient
g	Breadth of the approved label
h	Strength of the supporting evidence base
i	Fit with national guidelines
j	Cost to the health system
k	Cost to the patient

ASK FOR EACH THERAPY IN TOP3

Q20. What, if anything, limits your use of [THERAPY]?

Multi code, maximum four. RANDOMISE codes 1 to 15. Anchor codes 16 and 17 last, in that order. Code 17 is exclusive.

- Tolerability or adverse event profile.....1
- Risk of interstitial lung disease specifically.....2
- Reimbursement or formulary restrictions.....3
- Prior authorisation burden.....4
- Out-of-pocket cost to the patient.....5
- Insufficient long-term outcome data.....6
- Lack of data in my specific patient population.....7
- Administration burden on the clinic or infusion suite.....8
- Required biomarker testing is not routine in my setting.....9
- Turnaround time for the required biomarker test.....10
- Guideline positioning does not favour it.....11
- Availability or supply constraints.....12
- Limited experience managing its adverse events.....13
- Patient preference against this mode of administration.....14
- Concern about drug interactions in my patient population.....15
- Something else.....16

Nothing limits my use.....17

ASK FOR EACH THERAPY IN TOP3 WHERE Q20 = 1 OR 2

Q21. You mentioned tolerability limits your use of [THERAPY]. Which specific adverse events are of most concern to you?

Open text, minimum 10 characters. FLAG THIS VERBATIM FOR PHARMACOVIGILANCE REVIEW. See Appendix E.

ASK FOR EACH THERAPY IN TOP3

Q22. Compared with 12 months ago, is your use of [THERAPY] increasing, staying the same or decreasing?

Single code.

Increasing significantly.....	1
Increasing slightly.....	2
Staying about the same.....	3
Decreasing slightly.....	4
Decreasing significantly.....	5

ASK FOR EACH THERAPY IN TOP3 WHERE Q22 = 1, 2, 4 OR 5

Q23. What is driving that change?

Open text, minimum 10 characters.

ASK FOR EACH THERAPY IN TOP3

Q24. If [THERAPY] were unavailable tomorrow, what would you use instead for those patients?

Single code dropdown. Show the therapies selected at Q13 in ORDER_Q13, excluding the current [THERAPY]. Add No systemic therapy coded 97.

END OF LOOP. Continue to Section F.

Section F. Attribute trade-off, MaxDiff

ASK ALL WHO REACH THIS SECTION

Q25. In the next few screens you will see groups of factors that might influence your choice of therapy. In each group, please select the one factor that is most important to you and the one that is least important.

MaxDiff exercise. 32 attributes, 14 screens, 5 attributes per screen. Use the balanced incomplete block design supplied as MT2026208_MAXDIFF_DESIGN.CSV. Do not generate the design in the survey platform.

Each respondent is assigned one design version. Store the version number and the attribute set shown on every screen. Best and worst cannot be the same attribute. Both selections are required to continue. Record time on page per screen.

Code	Attribute
A1	Median overall survival benefit over current standard of care
A2	Median progression-free survival benefit
A3	Objective response rate
A4	Duration of response
A5	Proportion of patients achieving complete response
A6	Benefit specifically in patients with brain metastases
A7	Benefit specifically in patients with liver metastases
A8	Consistency of benefit across PD-L1 expression levels
A9	Rate of grade 3 or higher treatment-related adverse events
A10	Rate of immune-related adverse events
A11	Rate of treatment discontinuation due to toxicity
A12	Rate of interstitial lung disease or pneumonitis
A13	Rate of severe cutaneous reactions
A14	Cardiac safety profile
A15	Requirement for steroid premedication
A16	Oral administration rather than intravenous infusion
A17	Dosing frequency
A18	Duration of each infusion
A19	Ability to be administered in an outpatient setting
A20	Availability of a fixed-dose rather than weight-based regimen
A21	Availability of five-year follow-up data
A22	Number of patients in the pivotal trial
A23	Whether the pivotal trial included patients from my region
A24	Head-to-head data against the current standard of care
A25	Real-world evidence beyond the clinical trial population
A26	Inclusion in national treatment guidelines
A27	Strength of the guideline recommendation
A28	Requirement for a companion diagnostic test
A29	Turnaround time of the required companion diagnostic
A30	Monthly cost to the health system
A31	Out-of-pocket cost to the patient
A32	Availability of a patient assistance programme

Section G. Profile evaluation

ASK ALL

Q26. On the following screens you will see four descriptions of hypothetical therapies. Please read each carefully and answer the questions that follow. You will not be able to return to a description once you have moved on.

Display only. Show the low-resolution warning here if screen width is below 1024 pixels.

ROTATE the order of the four profiles across respondents using a Latin square. Store the position of every profile as PROFILE_POS_1 to PROFILE_POS_4. Each profile must be shown for a minimum of 25 seconds before the continue button appears. Record time on card per profile.

SHOW PROFILE 1 OF 4

Q27a. [Profile A: DISPLAY AS A SINGLE CARD]

MECHANISM: A bispecific antibody targeting PD-1 and VEGF, administered as an intravenous infusion.

INDICATION: First-line treatment of advanced or metastatic non-small cell lung cancer in patients with PD-L1 expression of 1 percent or above, regardless of histology.

EFFICACY: In a randomised phase III trial of 892 patients, median overall survival was 24.1 months compared with 18.6 months for pembrolizumab monotherapy, hazard ratio 0.74. Median progression-free survival was 11.2 months compared with 7.8 months, hazard ratio 0.68. Objective response rate was 52 percent compared with 39 percent.

SAFETY: Grade 3 or higher treatment-related adverse events occurred in 41 percent of patients compared with 28 percent in the comparator arm. The most common were hypertension in 12 percent, proteinuria in 8 percent and immune-related pneumonitis in 4 percent. Treatment discontinuation due to toxicity occurred in 14 percent of patients.

ADMINISTRATION: Fixed dose, intravenous infusion over 60 minutes, once every three weeks. No premedication required. Outpatient administration.

DIAGNOSTIC: Requires PD-L1 immunohistochemistry. No additional companion diagnostic.

COST: Approximately 14,200 per month to the health system. Patient assistance programme available.

Display only. Continue button hidden for the first 25 seconds.

SHOW PROFILE 2 OF 4

Q27b. [Profile B: DISPLAY AS A SINGLE CARD]

MECHANISM: An oral, third-generation tyrosine kinase inhibitor with central nervous system penetration.

INDICATION: First-line treatment of advanced non-small cell lung cancer with an activating EGFR mutation, including exon 20 insertions.

EFFICACY: In a randomised phase III trial of 614 patients, median progression-free survival was 22.6 months compared with 18.9 months for osimertinib, hazard ratio 0.79. Overall survival data are immature at the time of this profile. Intracranial objective response rate was 71 percent in patients with measurable brain metastases at baseline.

SAFETY: Grade 3 or higher treatment-related adverse events occurred in 33 percent of patients. The most common were rash in 9 percent, diarrhoea in 7 percent and paronychia in 5 percent. Interstitial lung disease of any grade occurred in 3.1 percent of patients, with two fatal cases reported. Dose reduction was required in 22 percent.

ADMINISTRATION: One tablet once daily, taken with or without food. No infusion visit required.

DIAGNOSTIC: Requires EGFR mutation testing by tissue or plasma. A companion diagnostic is approved alongside the therapy, with a stated turnaround of 7 to 10 days.

COST: Approximately 11,800 per month to the health system.

Display only. Continue button hidden for the first 25 seconds.

SHOW PROFILE 3 OF 4

Q27c. [Profile C: DISPLAY AS A SINGLE CARD]

MECHANISM: An antibody-drug conjugate targeting TROP-2, administered as an intravenous infusion.

INDICATION: Second-line and later treatment of advanced non-small cell lung cancer in patients who have progressed on platinum-based chemotherapy and an immune checkpoint inhibitor, regardless of biomarker status.

EFFICACY: In a randomised phase III trial of 520 patients, median overall survival was 12.4 months compared with 9.1 months for docetaxel, hazard ratio 0.71. Objective response rate was 29 percent compared with 14 percent. Median duration of response was 7.9 months.

SAFETY: Grade 3 or higher treatment-related adverse events occurred in 56 percent of patients. The most common were neutropenia in 31 percent, stomatitis in 11 percent and diarrhoea in 9 percent. Interstitial lung disease of any grade occurred in 5.4 percent. Prophylactic granulocyte colony stimulating factor is recommended.

ADMINISTRATION: Weight-based dose, intravenous infusion over 3 hours for the first dose and 1 hour thereafter, on days 1 and 8 of a 21-day cycle. Premedication with corticosteroid and antihistamine required.

DIAGNOSTIC: No companion diagnostic required.

COST: Approximately 16,500 per month to the health system.

Display only. Continue button hidden for the first 25 seconds.

SHOW PROFILE 4 OF 4

Q27d. [Profile D: DISPLAY AS A SINGLE CARD]

MECHANISM: An oral KRAS G12C inhibitor given in combination with an intravenous anti-PD-1 antibody.

INDICATION: First-line treatment of advanced non-small cell lung cancer with a KRAS G12C mutation.

EFFICACY: In a single-arm phase II trial of 148 patients, objective response rate was 63 percent and median progression-free survival was 13.4 months. No randomised comparison is available. Accelerated approval is anticipated, with a confirmatory phase III trial ongoing.

SAFETY: Grade 3 or higher treatment-related adverse events occurred in 47 percent of patients. Hepatotoxicity of any grade occurred in 38 percent, with grade 3 or higher in 16 percent. Regular liver function monitoring every two weeks for the first three months is required.

ADMINISTRATION: Oral tablet twice daily, plus an intravenous infusion once every four weeks.

DIAGNOSTIC: Requires KRAS G12C testing, typically as part of an NGS panel.

COST: Approximately 19,300 per month to the health system for the combination.

Display only. Continue button hidden for the first 25 seconds.

ASK AFTER EACH PROFILE

Q28. Based on this description, for what proportion of your advanced NSCLC patients would you consider this therapy?

Slider, 0 to 100, no default position. The respondent must move the slider before continuing, so that zero is a deliberate answer rather than an unmoved control.

ASK AFTER EACH PROFILE

Q29. Compared with the therapy you currently use most often, [PIPE: TOP3_1], how would you rate this profile?

Single code, 5-point scale. If TOP3_1 is empty, replace the piped text with the phrase your current standard of care.

Much worse.....	1
Somewhat worse.....	2
About the same.....	3
Somewhat better.....	4
Much better.....	5

ASK AFTER EACH PROFILE

Q30. What is the single most appealing aspect of this profile?

Open text, minimum 10 characters.

ASK AFTER EACH PROFILE

Q31. And what concerns you most about this profile?

Open text, minimum 10 characters. FLAG FOR PHARMACOVIGILANCE SCREENING, see Appendix E.

ASK AFTER ALL FOUR PROFILES HAVE BEEN SHOWN

Q32. Thinking about all four profiles you have seen, which would you be most likely to prescribe, and which least likely?

Two single-code questions on one screen. Show the four profiles by name in the order they were displayed to this respondent. Most and least cannot be the same profile.

Section H. Message evaluation

ASK ALL

Q33. Below are a number of statements that might be made about a new therapy for advanced NSCLC. For each, please indicate how persuasive you find it.

Grid, one row per message. RANDOMISE row order. Scale 1 to 7 where 1 is Not at all persuasive and 7 is Extremely persuasive. Force a response on every row.

M1

In a head-to-head phase III trial, this therapy extended median overall survival by 5.5 months compared with the current first-line standard of care.

M2

This therapy achieved a 71 percent intracranial response rate in patients with brain metastases, a population where treatment options remain limited.

M3

A once-daily oral tablet removes the need for infusion visits, reducing time in the clinic for patients who are already managing a substantial treatment burden.

M4

Five-year follow-up data are now available, showing that 28 percent of patients remain alive and progression free.

M5

The companion diagnostic returns results within 7 to 10 days, so treatment decisions need not wait on testing.

M6

Benefit was consistent across PD-L1 expression levels, including patients with PD-L1 below 1 percent.

M7

The patient assistance programme caps monthly out-of-pocket cost, and enrolment is completed by the clinic in under ten minutes.

M8

This therapy is included in the most recent national guideline update as a category 1 preferred first-line option.

ASK ALL

Q34. Which two of these statements would most influence your prescribing decision?

Multi code, exactly two selections. Show the messages in the same randomised order used at Q33.

ASK IF ANY MESSAGE RATED 6 OR 7 AT Q33

Q35. You rated [PIPE: HIGHEST RATED MESSAGE] highly. What makes that statement persuasive to you?

Open text. If two or more messages tie at the highest rating, pipe the one appearing first in the respondent's randomised order.

Section I. Guidelines and information sources

ASK ALL

Q36. Which of the following sources do you rely on when deciding how to treat advanced NSCLC?

Multi code. RANDOMISE codes 1 to 20. Anchor code 21 last, exclusive.

National treatment guidelines.....	1
International treatment guidelines.....	2
Peer-reviewed journal articles.....	3
Congress presentations attended in person.....	4
Congress presentations viewed online.....	5
Tumour board or multidisciplinary team discussion.....	6
Colleagues within my institution.....	7
Colleagues outside my institution.....	8
Key opinion leader lectures.....	9
Pharmaceutical company medical science liaisons.....	10
Pharmaceutical company sales representatives.....	11
Sponsored symposia.....	12
Continuing medical education modules.....	13
Clinical trial protocols I am involved in.....	14
Institutional treatment pathways.....	15
Payer or formulary communications.....	16
Medical news websites.....	17
Podcasts or webinars.....	18
Social media discussion among clinicians.....	19
Patient advocacy group materials.....	20
None of these.....	21

ASK IF TWO OR MORE SOURCES SELECTED AT Q36

Q37. And which three of these are most influential?

Ranking, exactly three ranks. Show only sources selected at Q36, in the Q36 display order.

ASK ALL

Q38. Which treatment guidelines do you follow most closely?

Single code. US: show codes 1 to 4. GERMANY: show codes 2, 3, 5, 6. JAPAN: show codes 2, 3, 7, 8. Code 9 shown in all markets.

NCCN Clinical Practice Guidelines.....	1
ESMO Clinical Practice Guidelines.....	2
ASCO Guidelines.....	3
Institutional pathway based on NCCN.....	4
Onkopedia Leitlinien.....	5
AWMF S3 Leitlinie Lungenkarzinom.....	6
Japanese Lung Cancer Society Guidelines.....	7
Institutional pathway based on J LCS.....	8
I do not follow a specific guideline closely.....	9

ASK ALL

Q39. How often does a payer or formulary restriction prevent you prescribing the therapy you consider clinically optimal?

Single code, 5-point frequency scale.

Always.....	1
Often.....	2
Sometimes.....	3
Rarely.....	4
Never.....	5

ASK IF Q39 = 1 TO 3

Q40. Which therapies are most often affected by these restrictions?

Multi code, maximum three. Show the therapies selected at Q13 in ORDER_Q13.

Section J. Unmet need

ASK ALL

Q41. Thinking about the treatment of advanced NSCLC today, where do you see the greatest unmet need?

Open text, minimum 20 characters. FLAG FOR PHARMACOVIGILANCE SCREENING, see Appendix E.

ASK ALL

Q42. In which of the following patient segments do you feel current treatment options are least adequate?

Multi code, maximum three. Show only segments where the respondent entered a value above zero at Q1.

ASK ALL

Q43. If one improvement could be made to the therapies available to you today, what would it be?

Open text, minimum 20 characters.

ASK IF Q1L IS GREATER THAN 20

Q44. You indicated that a substantial share of your patients have unknown or untested biomarker status. How does that affect your first-line treatment choice?

Open text. Note the condition refers to a value inside the Q1 grid, not to a separate question.

Section K. Patient dialogue

ASK ALL

Q45. How often do patients raise a specific therapy by name during consultation?

Single code, 5-point frequency scale.

- Always.....1
- Often.....2
- Sometimes.....3
- Rarely.....4
- Never.....5

ASK IF Q45 = 1 TO 3

Q46. Which therapies do patients most often raise?

Multi code, maximum three. Show the therapies selected at Q13 in ORDER_Q13, plus a code 96 for a therapy not on this list.

ASK ALL

Q47. How much influence does a patient's stated preference have on your final treatment decision?

Single code, 5-point scale. NOTE: wording amended at v3.1 after the pilot, where the earlier phrasing was read as asking about compliance rather than influence.

- A great deal..... 1
- Quite a lot.....2
- A moderate amount.....3
- A little.....4
- None at all.....5

ASK ALL

Q48. How often does out-of-pocket cost to the patient change which therapy you prescribe?

Single code, 5-point frequency scale. JAPAN: precede with the clarification that this refers to the patient co-payment portion.

- Always.....1
- Often.....2
- Sometimes.....3
- Rarely.....4
- Never.....5

Section L. Treatment sequencing

ASK ALL

Q49. For a newly diagnosed patient with PD-L1 expression of 50 percent or above and no actionable driver alteration, what sequence would you most likely follow?

Three single-code dropdowns on one screen, labelled First line, Second line and Third line. Each dropdown contains the therapies selected at Q13 in ORDER_Q13, plus code 96 for a therapy not listed, code 97 for no systemic therapy and code 98 for I would not plan that far ahead.

The same therapy may be selected in more than one dropdown only where it would genuinely be continued. Show a soft check if the same code is chosen for first and second line.

ASK ALL

Q50. For a newly diagnosed patient with an EGFR common activating mutation, what sequence would you most likely follow?

As Q49. Ask only if Q1a is greater than zero, otherwise skip.

IF Q1A = 0, SKIP TO Q51.

ASK ALL

Q51. What typically prompts you to change a patient's systemic therapy?

Multi code, maximum four. RANDOMISE codes 1 to 10. Anchor code 11 last.

Radiographic progression on imaging.....	1
Clinical deterioration ahead of imaging.....	2
Unacceptable toxicity.....	3
Patient request to stop or change.....	4
Completion of a planned fixed duration.....	5
Rising tumour markers.....	6
New brain metastases specifically.....	7
Results of repeat biomarker testing.....	8
A new therapy becoming available or reimbursed.....	9
Tumour board recommendation.....	10
Something else.....	11

ASK ALL

Q52. In your practice, what is the median time a patient remains on first-line systemic therapy for advanced NSCLC, in months?

Numeric, one decimal allowed, range 0.5 to 60. Add a Don't know checkbox coded 98.

ASK ALL

Q53. How often do you obtain a repeat biopsy or liquid biopsy at the point of progression?

Single code, 5-point frequency scale.

Always.....	1
Often.....	2
Sometimes.....	3
Rarely.....	4
Never.....	5

ASK IF Q53 = 4 OR 5

Q54. What prevents repeat testing at progression?

Multi code, maximum three. Show the same list as Q9, in a newly randomised order. Store the new order separately as ORDER_Q54.

ASK ALL

Q55. How has your typical treatment sequence for advanced NSCLC changed over the past two years?

Open text, minimum 20 characters.

Section M. Clinic operations and resource use

ASK ALL

Q56. In a typical week, approximately how many hours do you personally spend on each of the following activities related to advanced NSCLC patients?

Numeric grid, one row per activity, hours to one decimal. Each row 0 to 60. Show a running total but do not constrain it. Soft check if the total exceeds 80 hours.

Row	Activity	Variable
a	Direct consultation with patients	Q56a
b	Reviewing biomarker and imaging results	Q56b
c	Tumour board or multidisciplinary meetings	Q56c
d	Prior authorisation and reimbursement paperwork	Q56d
e	Managing adverse events, including unscheduled contacts	Q56e
f	Discussing treatment options with patients and families	Q56f
g	Clinical trial screening and enrolment	Q56g
h	Keeping up with new clinical evidence	Q56h

ASK ALL

Q57. How often does infusion chair or day unit capacity influence which therapy you select?

Single code, 5-point frequency scale.

- Always.....1
Often.....2
Sometimes.....3
Rarely.....4
Never.....5

ASK IF Q57 = 1 TO 3

Q58. In what way does capacity influence the decision?

Open text, minimum 10 characters.

ASK ALL

Q59. For patients on oral targeted therapy, how do you monitor adherence?

Multi code. RANDOMISE codes 1 to 7. Anchor code 8 last, exclusive.

- Patient self-report at each visit.....1
Pill counts.....2
Pharmacy refill records.....3
Specialty pharmacy reporting.....4
Nurse-led telephone follow-up.....5
A digital adherence application.....6
Family or caregiver report.....7
We do not formally monitor adherence.....8

ASK ALL

Q60. How many oncology nurses support your advanced NSCLC patients at your site?

Numeric, 0 to 100. Add a Don't know checkbox coded 98.

ASK ALL

Q61. Which one change would most reduce the operational burden of treating advanced NSCLC in your setting?

Open text, minimum 20 characters.

Section N. Respondent profile

ASK ALL

D1. Approximately what percentage of your working time is spent in direct patient care?

Numeric, 0 to 100.

IF D1 < 20, FLAG AS LOW CLINICAL INVOLVEMENT, CODE 51, BUT RETAIN THE RESPONDENT.

ASK ALL

D2. Have you participated in a clinical trial as an investigator in the last three years?

Single code.

- Yes, as principal investigator..... 1
- Yes, as a sub-investigator..... 2
- No..... 3

ASK ALL

D3. Approximately how many physicians treat lung cancer at your institution, including yourself?

Numeric, 1 to 200.

ASK ALL

D4. In which region of your country do you primarily practise?

Single code. US: census regions. GERMANY: Bundesland list. JAPAN: prefecture grouping. Lists supplied separately as MT2026208_REGIONS.XLSX.

ASK ALL

D5. What is your gender?

Single code.

- Male..... 1
- Female..... 2
- Prefer to self-describe..... 3
- Prefer not to say..... 4

ASK ALL

D6. Would you be willing to be recontacted for a follow-up interview about this topic?

Single code. If yes, do not capture contact details here. The panel provider handles recontact against the respondent ID.

- Yes..... 1
- No..... 2

Appendix A. Derived variables

Variable	Derivation	Used at
YEARS_PRACTICE	2026 minus S3	S3 terminations
ORDER_Q13	Randomised display order of the 40-item therapy list	Q14, Q15, Q16, Q24, Q40, Q46
TOP3_1 to TOP3_3	Three highest-share therapies at Q15, ties broken by ORDER_Q13	Section E loop, Q29
LOOPCOUNT	Number of therapies in TOP3, 1 to 3	Loop control, progress display
BIOMARKER_COMPLETE	Value entered at Q8	Q9 display condition
MD_VERSION	MaxDiff design version assigned	Section F analysis
PROFILE_POS_1 to 4	Latin square position of each profile	Section G analysis
HIGHEST_MSG	Highest-rated message at Q33, ties broken by display order	Q35 piping
SOFTCHECK_S6	Set if the respondent confirmed a volume above 120 at S6	Data cleaning
SEED	Randomisation seed assigned at survey start	Audit reconstruction

Appendix B. Grids requiring a total of 100

Question	Rows	Rule
Q1	12 patient segments	Must total exactly 100
Q2	4 lines of therapy	Must total exactly 100
Q15	Therapies prescribed at Q14	Must not exceed 100, need not equal it

Appendix C. Piping map

Source	Destination	Rule
Q13 selections	Q14 list	Same items, same order
Q14 selections	Q15 grid rows	Same items, same order
Q15 derived TOP3	Section E loop	Descending share order
TOP3_1	Q29 stem	Fallback text if empty
Q1 non-zero rows	Q16 rows, Q18, Q42	Only segments above zero
Q3 selections	Q17 list	Only lines the respondent decides on
Q33 highest rating	Q35 stem	Ties broken by display order
Q36 selections	Q37 ranking list	Same items, same order

Appendix D. Termination and disposition codes

Code	Reason	Type
10	Specialty out of scope at S1	Hard terminate
11	Not board certified at S2	Hard terminate
12	Years in practice out of range at S3	Hard terminate
13	Patient volume below threshold at S6	Hard terminate
14	Does not personally prescribe, S5 or S7	Hard terminate
15	Conflict of interest at S8	Hard terminate
16	No therapy awareness at Q13	Hard terminate
17	Practice setting out of scope at S4	Hard terminate
18	Recent participation in similar research at S9	Hard terminate
19	Consent not given at S10	Hard terminate
30	Specialty by market quota full	Quota full

Code	Reason	Type
40	Abandoned mid-loop	Partial
41	Abandoned during MaxDiff	Partial
42	Abandoned during profile evaluation	Partial
51	Low clinical involvement at D1, retained	Complete, flagged
52	Speeder, LOI below one third of median, retained for review	Complete, flagged
99	Complete	Complete

Appendix E. Adverse event handling

Q21, Q31 and Q41 are open-ended questions where a respondent may describe an adverse event. All three must be screened.

- Every verbatim from these questions is exported daily to a separate secure file, regardless of whether the respondent completed the survey.
- A verbatim is flagged if it names any Meridian product together with any clinical event term from the supplied MedDRA subset, MT2026208_MEDDRA_SUBSET.XLSX.
- Flagged verbatims are sent to the client safety mailbox within 24 hours of collection, including weekends.
- The respondent is not contacted or re-identified. Reports are submitted with the respondent ID only.
- This process continues for the full period the study is in field and for 30 days after close.
- A nil return is sent on any day where no verbatim is flagged, so that the absence of a report is never ambiguous.

Appendix F. Data export map

The delivered data file must contain the following in addition to the answer variables for every question.

Variable	Type	Description
RESPID	Text	Unique respondent identifier supplied by the panel
MARKET	Code	1 US, 2 Germany, 3 Japan
LANG	Code	Language version served
SEED	Integer	Randomisation seed, required to reconstruct the survey for audit
ORDER_Q7	Text	Display order of the test list at Q7, comma separated codes
ORDER_Q13	Text	Display order of the 40-item therapy list
ORDER_Q20_i	Text	Display order of the barrier list at Q20, one variable per loop iteration
ORDER_Q36	Text	Display order of the information source list
ORDER_Q54	Text	Display order of the repeat barrier list at Q54
MD_VERSION	Integer	MaxDiff design version
MD_SCREEN_1 to 14	Text	Attribute codes shown on each MaxDiff screen
PROFILE_POS_1 to 4	Integer	Display position of each profile
T_PROFILE_1 to 4	Seconds	Time on card for each profile
T_MD_1 to 14	Seconds	Time on page for each MaxDiff screen
LOOP_ITER	Integer	Loop iteration number on every Section E variable
LOOP_THERAPY	Code	Therapy code for the current loop iteration
SOFTCHECK_S6	Flag	Set if a volume above 120 was confirmed at S6
SOFTCHECK_Q4	Flag	Set if new starts exceeded total patients
SOFTCHECK_Q56	Flag	Set if weekly hours exceeded 80
LOI_TOTAL	Seconds	Total interview length
TERM_CODE	Code	Termination or disposition code for every record, including completes
TERM_POINT	Text	Question identifier at which the session ended

Appendix G. Translation and market variation notes

1. Response code values are identical across all three markets. Under no circumstances renumber codes to suit a local list.
2. Where a question carries a market-specific instruction, that instruction appears in the question itself and overrides the base wording for that market only.
3. S2 uses Facharzt in Germany and the local specialist certification term in Japan.
4. Q38 shows a different guideline list per market. The codes are drawn from a single master list, so a code absent in a market is simply not displayed.
5. D4 uses a different region list per market, supplied separately.
6. Currency figures in Section G profiles are shown in USD, EUR and JPY respectively. The values are not converted at spot rate, they are the client-supplied local price points and must be used exactly as given.
7. The consent wording at S10 is legally reviewed per market and must not be translated from the English.
8. Japanese script must not be truncated in grid column headers. Where a header does not fit, use the abbreviated form supplied in the translation file rather than shortening it in the platform.